
LYFE PLASTICS AND DERMATOLOGY

Getting her in front of patients, in six weeks

A hundred and seventy names produced nothing. This is why, and what to do about it before she travels.

CONSULTATIONS HELD, SIX WEEKS

73 to 109

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A hundred and seventy leads, eleven warm, nothing closed.

That is a warm rate of 6.5 per cent against a benchmark band of 20 to 35 per cent. The practice is running at between a sixth and a quarter of normal, which is not the kind of gap that creative explains.

A Nigerian practice published 392 consultations producing 245 operations. Closing is not this market's problem.

	March 2026	Benchmark
Leads	170	
Reached the warm stage	11 (6.5%)	20% to 35%
Consultations held	Not recorded	
Cases	0	46% of consultations, median



It is a speed problem or an audience problem. One day of digging says which.

- **The number is a quarter of normal, not a bit under** Benchmarks put lead to booked consultation at 20 to 35 per cent for cold paid social. March ran at 6.5 per cent. Nothing about creative or offer explains a gap that size on its own.
- **Nigerian consultations close at 62 per cent** A Nigerian practice published 392 consultations producing 245 operations. The closing is not the problem in this market. The leads never became consultations.
- **So it is one of two things, and they have different fixes** Either nobody rang them fast enough and often enough, or the campaign bought the wrong people. Both are cheap to test and one of them is free to fix.
- **Three questions settle it inside a day** What was the actual time from form to first call. How many times was each lead called before it was abandoned. What did the page the ads pointed at weigh on a Lagos 4G connection.

If the answer is speed, the fix costs nothing and starts on Monday. Find out before spending a naira on media.



The clock is the campaign.

Calling at five minutes rather than thirty multiplies the odds of qualifying a lead by twenty one. The median company takes forty two hours and 23 per cent never respond at all.

AND THEN KEEP CALLING

- One call reaches 39 per cent of leads. Two reaches 72 per cent.
- 93 per cent of every lead that ever converts is reached inside six attempts.
- Half of all leads are never called a second time.
- A lead that takes seven or more calls to reach is 45 per cent less likely to convert.

Velocify, 3.5 million leads from 400+ companies

First call placed at	Lift in conversion
1 minute	+391%
2 minutes	+160%
30 minutes	+62%
1 hour	+36%
5 hours	+24%
24 hours	+17%



Six calls, five emails, twenty two days. Printed and on the wall.

When	Time	Channel	Worth
Day 1	0 to 1 minute	Call	+156%
Day 1	About 20 minutes	Email	+49%
Day 1	30 to 60 minutes	Call	+58%
Day 1	1 to 2 hours	Call	+25%
Day 4	Morning	Email	+85%
Day 5	Morning	Call	+22%
Day 8	Morning	Email	+52%
Day 14	Morning	Call	+23%
Day 15	Morning	Call and email	+9% and +37%
Day 22	Morning	Email	+44%

Six calls, five emails, twenty two days. Not more than five emails in the first month, because the study found that a sixth drops conversion by 36 per cent. Correct call timing is worth 49 per cent on its own, correct email timing 53 per cent, and the two together 128 per cent.



Five regulators, and the one everybody forgets is the one with the fine attached.

	Bites on	Exposure	What it says	What we do
ARCON	Every paid item	MINIMUM NGN 500,000 PER UNAPPROVED AD	The 2022 Act defines an advertisement to include an event or a person, on any medium. Everything needs pre exposure approval, only a licensed agency may submit it, and the liability sits on the advertiser. Influencer posts are covered.	Budget the accelerated tier at 4, 8 or 16 working hours. This is a critical path line item, not an afterthought.
NAFDAC	Naming any product	Cannot say the brand names at all	Toxin and fillers are prescription only medicines, advertisable only in medical journals. Cosmetics need separate approval. No consumer promotions at all on anything medicinal.	No brand names anywhere in public copy, and no book in October offer on injectables. The published list already uses generic names only.
MDCN, Part F	Who may be promoted	A disciplinary offence for the doctor	Self advertisement by a practitioner is misconduct, as is canvassing and as is being associated with those who sanction it. Permitted: public health education, a facility describing its own services, leaflets inside the premises.	The clinic advertises. She educates. That single configuration satisfies all of it, and it is what the page and the creative already do.
MDCN, October 2025	Social media conduct	New, and directly on point	Issued 14 October 2025. No identifiable patient image without explicit written consent. Personal social accounts are not to be used for consultations, which kills any message me to book mechanic.	One clinic WhatsApp line, never a personal account. No patient images in this window at all.
MDCN registration	Whether she can see anyone	The one that decides the rest	No practice without registration. Temporary registration needs a letter of good standing and a firm offer of employment from a Nigerian hospital, so the clinic acts, not her. Jurisdiction reaches only registered practitioners, so the exposure lands on the clinic and its medical director.	Start it now and say so. Registration in progress is a credibility asset.



Do not market a clinic date we cannot legally staff.

A foreign practitioner needs a Special Temporary Permit or Temporary Limited Registration from the Council before carrying out medical activity in Nigeria. Separately, a medical mission must be notified to the Diaspora Unit of the Federal Ministry of Health three months in advance. Nobody has established which of those two a commercial visiting clinic is read as. If it is the second, a November clinic date has already passed its notice period.

This is the critical path. Everything in this deck is contingent on it and nothing else in it is.

- **This week.** Call the Council and get the position in writing. Not an opinion from a lawyer who has not called them.
- **Until then.** Everything says she sets the standard and the clinic's own registered clinicians treat. The page is already built that way, so nothing has to be retracted.
- **The upside.** A virtual consultation is a real consultation. A study of 1,889 new plastic surgery patients found no difference in whether they went on to have a procedure. She can qualify a diary from abroad while the registration is in train.



The copy rules, on one page, so nobody has to remember the reasoning.

The clinic advertises. She educates. Every line on the left is sayable because it describes a business role or a factual American credential. Every line on the right holds her out as a practitioner available to treat Nigerian patients, which she is not yet registered to be.

Say this	Never this
Founder, or co-founder, of the practice	Our plastic surgeon
Promoter, programme director, clinical adviser	Now accepting patients in Lagos
United States board certified plastic surgeon	Book your surgery with her in Lagos
An educational series with Dr Chinwe Kpaduwa	Message her to book a consultation
The clinic describes its services	She describes her skill, experience or results
In person clinics planned, subject to clearance	Any date certain promise of surgery

In 2020 a Beverly Hills plastic surgeon was sued for posting patient photographs without consent. A judgment of six hundred thousand dollars against him was upheld on appeal in May 2023. Her existing before and after images were taken under United States consents that will not cover Nigerian marketing. Re papering them is a lawyer's job, not a designer's, and in this window we simply do not use them.



There are about a hundred and twenty four plastic surgeons for two hundred and thirty five million people.

Consultant plastic surgeons in Nigeria	about 124	One per 1.5 million people
What the British association recommends	one per 100,000	We are at a fifteenth of it
Dermatologists in Nigeria	about 90	For 235 million people

This is the strongest structural argument in the business and nobody is using it. The constraint in Lagos aesthetics is not demand and it is not capital. It is that there are almost no qualified people.



We are not competing with Lagos. We are competing with Istanbul, and we already win on price.

	All in	What is in the number
Turkey, tummy tuck and buttock augmentation, everything in	NGN 10.8m to 14.9m	Surgeon, hospital, hotel, transfers, garments, a coordinator and a WhatsApp line, in one number
Lagos, the published equivalent	NGN 5.1m	Surgery only. The rest arrives later and separately

Turkey is not cheaper. It is twice the price and it wins anyway, because it sells one number, a cover story and aftercare that is already arranged. We should copy the packaging, not the price.



The operation is not the expensive part. Nobody in Lagos prices the rest.

One Lagos patient published every line of what she spent. It is the most useful document in this market and it says the same thing on every line: the quote was for the surgery, and the surgery was the cheap half.

It only works if it is true. Cost it and staff it in week one, or take it off the page.

	NGN m	What it was
The operation	8.00	Deposit, theatre, room, pre operative tests, flights
Everything after it	3.86	Twenty massages, thirty nights of accommodation, garments, medication, scar creams, a carer for six weeks
What she actually spent	16.00	Including the complications nobody budgets for

Aftercare cost her more than the surgery did, and none of it was in the quote. A named, included, priced aftercare programme is the clearest piece of open ground in this market, and it is what Istanbul is actually selling.



This market does not need convincing that surgery is desirable. It needs convincing that it is survivable.

When	What the market remembers
October 2005	Stella Obasanjo dies two days after liposuction in Spain. The surgeon is later convicted of negligent homicide. It is still the first thing a Nigerian search returns.
May 2024	A Lagos cosmetic surgeon is convicted over a patient's death. The sentence is one year, with the option of a fine of one hundred thousand naira.
January 2026	A Lekki operator who studied nursing, not medicine, is exposed after one death and more than thirty botched procedures. He recruited through an Instagram giveaway and a WhatsApp group.
March 2026	A patient dies after a revision procedure at a registered Lagos clinic, following five hours in an ambulance with no intensive care bed available.

This market does not need persuading that surgery is desirable. It needs persuading that it is survivable. Safety is not a brand value here, it is the entire proposition, and it has to be made of facts rather than adjectives.



American standard, close to home is taken. Do not fight for it.

What he owns	A Nigerian American, double board certified, on Victoria Island since well before us. His line is world class care, now close to home, and American standard plastic surgery in Africa. He has had a decade to own it.
What he does not do	No before and after gallery, no testimonials, no published prices. The whole proposition is credential arbitrage and a safety contrast against untrained operators. That restraint is itself the lesson.
Where we do not fight	American standard, close to home. It is taken, and arguing about whose board is better is a losing conversation.
Where we win	A woman operating on women. Darker skin as a stated competence rather than an afterthought. Restraint instead of transformation. Craniofacial depth. And a publicly argued position against the excess that this market is frightened of.



She is a credentials asset, not a distribution asset.

	Scale	
YouTube	2,250 subscribers, 303 videos, 360,088 lifetime views	Recent videos run in the tens and low hundreds. It is a library, not a channel.
Instagram, personal	about 898 followers	Lagos is already in her bio.
Instagram, the practice	about 398 followers	Near dormant.
LinkedIn	about 1,000 followers	Professional, not patient facing.
Nigerian press	Nothing	No coverage of any kind, which means a Nigerian launch is genuinely first run rather than a correction.

Her total reachable audience is somewhere under four thousand people and almost all of it is American. This campaign cannot run on her following and should stop pretending it might. It runs on borrowed audience: the clinic's own book, partners' rooms, the press and referring doctors. She is a credentials asset, not a distribution asset.



Three hundred and three videos already exist, in her voice, arguing our thesis.

- **Three hundred and three videos, already shot** In her own voice, on her own thesis, already edited. The single biggest asset in the business and it needs re cutting for a Nigerian audience, not re shooting. This is weeks of publishing available at the cost of an editor.
- **A thesis that is already the right one** Her own titles: the best plastic surgery is not obvious, the business of insecurity, doctors should not be influencers, why I will never break ribs for beauty, the biggest skincare mistake women of colour make. We did not invent the art of looking like yourself. She has been arguing it for two years.
- **A monthly live format that already exists** Her own recurring programme, already branded, already running. It can be run once in the window at a Lagos friendly hour. It is testimonial based, so it runs with her existing patients and no Nigerian clinical claim.
- **Booking infrastructure that already works** A live calendar with five configured appointment types, including virtual.



Twelve years of teaching and building in West Africa, never once used.

2004	Founded and led the Nigerian Student Association at Harvard, and ran a conference for a hundred and fifty young Nigerian Americans on their role in Nigeria.
2011	Taught free flap reconstruction at Korle Bu teaching hospital in Accra, as an instructional course lecturer.
2014	Co founded a partnership dedicated to improving healthcare delivery, medical education and hospital quality in Nigeria. Still running.
2016	Gave Grand Rounds at Boston Medical Center titled Why Tunde Can't Operate, on surgical training in developing Africa. Her title, her position, a decade old.
Always	Speaks Igbo.

The profession's own word for a visiting team that operates and leaves is a surgical safari, and it is a live pejorative here. It is also the only serious objection the referrers will raise. She has a twelve year documented record of teaching and building in West Africa, and none of it has ever been used. Put it in the first paragraph of every approach to a doctor.



She teaches. She does not sell. That is forced on us, and it is also the better strategy.

What everyone else sells	Results, aspiration, a celebrity at the launch, and a price on request
What the market is frightened of	Not the cost. Who is holding the needle, and who answers the telephone at two in the morning
What we sell	Judgement. A consultation that will tell you not to. A published price. A named clinician. An aftercare programme that is in the quote.
Why it is also the only legal option	The Council permits education and forbids self promotion, so the compliant creative and the high trust creative are the same creative. That is lucky, and we should take the luck.



Instagram is a quarter the size everybody thinks, and it is shrinking.

Reach in Nigeria		
WhatsApp	54m users, on 95 per cent of Nigerian smartphones	96.5 per cent of Nigerian internet users. Click to message converts several times better than click to form, and Meta waives message fees for 72 hours after a click to WhatsApp ad. This is the channel, not a channel.
Facebook	38.0m addressable	Four times Instagram's reach here and nobody in this category is using it properly.
Instagram	10.0m addressable, and shrinking 6 per cent a year	54 per cent male. The assumption that Instagram is where affluent Lagos women are is half right and a quarter the size people think.
TikTok	47.8m addressable, growing 43 per cent	68 per cent male. Large, cheap and demographically wrong for this. Spill, not spend.

Nigeria has the lowest Meta cost per thousand in the world, about a dollar fifty against a global average of six fifty nine, and a twelve cent click. The same source warns that bot and junk traffic runs 20 to 30 per cent above tier one markets. Cheap clicks and dirty traffic is exactly the profile that produces 170 leads and 11 warm.



The page is built, and every decision in it traces to something measured.

- **Two doors, not one** An aesthetic consultation with the clinic's registered clinicians, and a surgical planning review that she reads herself. Different appointments, different forms, different promises.
- **No navigation on the page** Removing navigation from a mid funnel page measured 16 to 28 per cent in a controlled test. Every link goes to the form, WhatsApp or the telephone.
- **A three step form, contact details last** Three fields converts best and the sharpest drop is the fourth. A form this long has to be stepped, and the commitment escalates rather than arriving all at once.
- **Prices published** A room that has to ask assumes the worst, and the one Lagos competitor who publishes is winning the comparison by default.
- **Her own screening questions, on the surgical door** Height, weight, nicotine and whether weight is still moving. Taken straight from her practice's own coordinator manual, which makes the lead actionable the moment it lands.
- **Weight is a conversion decision** 86 per cent of Nigerian traffic is mobile at about 15 megabits, and data costs 575 naira a gigabyte. No hero video. A tenth of a second measured 8.4 per cent in conversions across thirty million sessions.



Eight rooms we can name, and not one of them needs a launch.

	What it is	What we do with it
GAIA Africa	A private members club for senior African women, clubhouse on Victoria Island, membership by application and recommendation	A private evening for 25. Highest concentration available, and partner sessions are a normal part of their programme.
WIMBIZ	3,757 associates. The 25th annual conference is 5 and 6 November at Eko Hotel, 2,600 or more women expected	The single densest room of the exact demographic in the whole window. It also sits right at the end of it, which is a timing problem worth solving now.
Medical Women's Association of Nigeria, Lagos	The Lagos branch reaches female general practitioners, obstetricians and dermatologists	A session for doctors. Continuing professional development is compulsory for licence renewal at 36 points every two years, so attendance is self motivated and education is the one promotion the Council permits.
The reformer pilates and barre studios	Lo Studio on Victoria Island, MOV Lab in Ikoyi, Spot Pilates	The best matched fitness segment there is. A consultation morning in the studio, hosted by the studio.
The salons	Bloom Hair Atelier, Tasala, The Nail Boutique across three sites	Where this audience already sits still for two hours and already discusses this subject. Cards on the counter and a consultation day.
Padel and the clubs	Four Ikoyi padel clubs, Ikoyi Club 1938 ladies sections, Lagos Country Club on the mainland	Category already takes brand sponsorship. Ladies sections, not the club at large.
Exquisite and Wellness and Style Live	An affluent female audience, a proven live format and a founder who sells partnerships	The best matched media partner in the market.
The podcast with the right room	Beyond with Ezinne, whose audience is explicitly Lagos style	Hyper niche shows convert faster than large general ones. Guest selection is reckoned to be half the return.



The evening on the tenth should be the first of seven things, not the only one.

A well run forty person open house produces fifteen to twenty five booked consultations. Six small formats of twenty five will out yield one evening of eighty, cost less in total, and spread the risk across venues and partners. The evening on the tenth should be the first of seven things, not the only one.

Twenty five people in a members club with a host who vouches for you is worth more than two hundred at a launch.

- **The host is the draw, not us.** A private evening works because it is somebody's room and somebody's guest list. We bring the conversation, they bring the people, and nobody is paid.
- **The doctors' session is different.** Continuing professional development is compulsory at 36 points every two years. A session for referring clinicians fills itself, is the one promotion the Council permits, and puts us in front of the people who send patients rather than the patients.
- **Everything terminates in the same place.** One WhatsApp number, one page, one coordinator, one queue. Seven rooms feeding seven different inboxes is how March happened.



The eleven are worth more than the other hundred and fifty nine put together.

	Count	What we do with them
The 11 who went warm	11	Worked individually, by telephone, on the phone put together.
The 159 who did not	159	One honest message, not a campaign. A writing that is not a discount.

Median time to re-engage a dormant contact is 57 days, and 75 per cent of reactivations land inside 89 days. Most of this list will answer after she has flown home. Plan it as January pipeline, not as a six week harvest.

What the list is actually worth

People the message reaches at all	20 to 65
Live conversations	8 to 25
Consultations booked	5 to 17
Cases, eventually	2 to 7



What happens when a Lagos prospect does the obvious thing and searches her name.

Anybody who hears her name and searches it finds a one star review on one American directory and two out of five on another, both from tiny samples, both American, and both above anything we publish. There is no Nigerian page about her at all. Either we publish enough high authority Nigerian facing material to outrank them inside the window, or the campaign runs as a closed loop with no search her name moment in it. Decide which, in week one.

And the two offers that contradict each other.

Her own practice site already offers Nigerians a free consultation and already says she sees Lagos patients virtually, from an address on Banana Island. The plan here prices a consultation at fifty thousand naira, which is what the market leader charges and what qualifies a lead. Both are live today. One of them has to go, and the free one is the one costing us the qualification.



Week one clears the gate and stops the leak. Everything else is sequencing.

1	6 to 10 Oct	Clear the gate and stop the leak	MDCN called and the registration position written down. The page live. The coordinator named, with the grid on the wall and a stopwatch on the queue. The 11 warm leads called personally. The evening on the tenth.
2	13 to 17 Oct	Open the doors	Consultation days running at capacity. Every guest from the evening called inside 48 hours. The 159 contacted once, honestly. Referrer visits begin, eight to twelve practices booked.
3	20 to 24 Oct	Take the rooms	First private evening at a members club. First studio or salon consultation morning. The doctors' session confirmed with a date. Education series publishing weekly.
4	27 to 31 Oct	Repeat what worked	Second and third small formats. Podcast and press land. The surgical review queue is read and answered. Second referrer round.
5	3 to 7 Nov	Convert and hand over	Fourth and fifth formats. WIMBIZ on the fifth and sixth. Training and sign off of the clinic team so the standard survives her leaving.
6	10 to 13 Nov	Close the window and open the next one	Final consultations. The December and January clinic block opened for booking while the diaspora is arriving. The report, and the brief for the next visit.

The window ends exactly where Detty December begins. Last year it brought 3.6 million people and 396 billion naira of spending, 55 per cent of it from the diaspora, with American arrivals overtaking British for the first time. A Nigerian American surgeon is precisely the profile that audience already trusts. These six weeks are the planting. December and January are the harvest, and the plan should be measured that way.



On these rates we run out of chairs before we run out of people.

	Target	
People worth calling, across all seven sources	99 to 207	On the rates in the previous slides, applied source by source
What the diary can actually hold	80	2 consultation days a week, 8 slots a day, 5 weeks. This is the binding constraint, not demand
Consultations held	73	At capacity, after the 8.9 per cent median no show a competently run practice sees
With a third consultation day	109	The same demand against 120 slots. This is the decision worth taking in week two
Surgical files she has read and answered	16 to 24	Written opinions, consented, with no date promised to anybody
Time from enquiry to first human contact	Under 1 hour, every time	The one number that decides whether any of the rest happens
Doctors who have met the team and hold the direct line	8 to 12	A first referral answered properly is the test. Flow comes next year

Both ends of the demand range clear the diary, so the question in week two is not how much media to buy. It is whether the clinic will give a third consultation day, which is worth 36 more consultations held and costs nothing in marketing.



One of these is worth more than the entire media budget.

- **A named coordinator, on the queue, inside the hour** Not a shared inbox and not a marketing person. One person whose job is the telephone, working the grid, measured on time to first contact. This single hire is worth more than the entire media budget.
- **The MDCN position in writing** What she can do, from when, and under which registration. Everything else is contingent on it.
- **WhatsApp Business Platform, not the free app** The free app caps broadcasts at 256 and only delivers to people who already saved the number, which means it cannot reach the 170 at all. The paid platform costs about five cents a message, so reaching the whole list costs under ten dollars.
- **A domain** The page cannot carry a management consultancy's web address on a plastic surgery flyer. This is a one day job and it is blocking the print.
- **Aftercare priced and staffed** The differentiator only works if it is real. Somebody has to cost the massages, the garments, the accommodation and the night line, and commit to them.
- **Decisions inside 48 hours** Six weeks does not survive a weekly approval cycle.



Five things could break this, and one of them is ours.

The registration does not come through in time	Then the surgical door closes and the aesthetic door carries the window. The page is already built so that she is the standard rather than the attending clinician, so nothing has to be unsaid.
We are pitching the direct competitor	The leading Lagos plastic surgery group, the only one publishing a surgical price list, is a live CFA marketing prospect. That is a conflict and it needs deciding before either piece of work goes further. It is also why none of this plan names a competitor.
The visiting surgeon reads as a safari	The profession's own term for a foreign team that operates and leaves. It is a live pejorative in Nigerian medical circles and it will cost us the referrers. The answer is a named local clinician, a named host facility, aftercare continuity in writing and a training component, all of which we are doing anyway.
Aftercare is promised and not delivered	This is the biggest reputational risk in the plan, because it is the one claim that is load bearing. If it cannot be staffed and costed in week one, take it off the page.
Scarcity gets invented	Quantity scarcity works and fabricated scarcity reverses, with documented anger and brand switching. If the diary is not actually finite, the page says nothing about slots. It is wired that way on purpose.

WHAT HAPPENS ON MONDAY

Three things, and none of them cost money.

- **Call the Council**

Get the registration position in writing. Everything else waits on it.

- **Name the coordinator**

One person, the telephone, the grid on the wall, and time to first contact measured from Monday.

- **Pull the March data**

Time to first call, number of attempts, and what the landing page weighed. One day of work settles the whole diagnosis.

